

Immunotherapy for melanoma

Immunotherapy uses the immune system to recognise and kill cancer cells.

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About immunotherapy for melanoma

The immune system protects the body against illness and infection. Immunotherapies are treatments that use the immune system to recognise and kill cancer cells. Two types of immunotherapy are used to treat [melanoma](#):

- checkpoint inhibitors drugs such as [ipilimumab](#) (YERVOY[®]), [pembrolizumab](#) (Keytruda[®]) and [nivolumab](#) (Opdivo[®])
- virus therapy called T-VEC (Imlygic[®]).

Your cancer doctor will explain what drugs are useful for you. They will give you information about how the drug is given and possible side effects.

Sometimes immunotherapy is given as part of a [clinical trial](#).

Contraception and fertility during treatment

Your doctor may ask you to use contraception to prevent pregnancy during and after some types of immunotherapy for melanoma. This is important if the treatment may harm a developing baby.

Some immunotherapy drugs can affect being able to [get pregnant](#) or [make someone pregnant](#). This is called your fertility. Your doctor or nurse can explain if your treatment is likely to cause fertility problems. If you are worried about this, talk to your doctor before your treatment starts.

Checkpoint inhibitor drugs for melanoma

Checkpoint inhibitor drugs activate white blood cells called lymphocytes to attack cancer cells. Lymphocytes are an important part of your immune system.

You may have checkpoint inhibitor drugs:

- After surgery to reduce the risk of melanoma coming back. This is called [adjuvant treatment](#). You usually have treatment for up to 12 months.

- To help slow the growth and help you live longer if melanoma cannot be removed with surgery or has spread ([advanced melanoma](#)). You may keep having treatment as long as it is working and are not causing difficult side effects.

You have the drug at a day unit as an outpatient. It is given as a drip into a vein. This is called intravenous infusion. Sometimes, 2 checkpoint inhibitor drugs are given together. Your cancer doctor or nurse will explain your treatment plan and what to expect.

After the treatment, you have a rest period of a few weeks. The immunotherapy and the rest period are called a cycle of treatment. The length of a cycle depends on the treatment you are having.

Checkpoint inhibitors that are used to treat melanoma include:

- [ipilimumab](#) (YERVOY[®])
- [nivolumab](#) (Opdivo[®])
- [pembrolizumab](#) (Keytruda[®]).

Possible side effects

Common side effects of checkpoint inhibitor drugs include:

- feeling tired
- skin changes or rashes
- diarrhoea
- feeling short of breath.

This type of drug affects the immune system and can cause side effects that are related to that. Your cancer doctor or nurse will give you information about this.

Side effects may be mild but sometimes they can become serious very quickly if they are not treated. Side effects can start during treatment. They can also start weeks, months, or sometimes more than a year, after you finish treatment.

It is important to tell your cancer doctor or nurse about any side effects you have as soon as possible. Side effects can happen after you have stopped having treatment.

Your doctor, nurse or pharmacist will also give you a card with information about your treatment on it. It is important to always have the card with you during treatment and after treatment finishes. You should always show it to any doctor or healthcare professional you need to see or who prescribes any other medication for you.

T-VEC for melanoma

T-VEC (Imlygic[®]) uses a virus that is injected into areas of melanoma that have spread to the skin or lymph nodes. It is only used if melanoma has not spread to other areas such as the bones or the brain.

This is a specialised treatment and may only be available at certain hospitals. Your doctor or nurse can give you more information.

The virus is designed to infect cancer cells. This helps your body's immune system find and destroy the area of melanoma that has been treated.

T-VEC is given by injection directly into the area of melanoma. You usually have this treatment every 2 weeks. Your cancer doctor will give you more information.

Some common side effects include:

- [tiredness](#)
- [tumour pain](#)
- redness or inflammation at the injection site
- flu-like symptoms
- [feeling sick](#)
- headaches.

Your cancer doctor will give you more information about side effects and managing them.

About our information

References



Below is a sample of the sources used in our melanoma information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Michielin O, van Akkooi ACJ, Ascierto PA, et al. Cutaneous melanoma: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. *Annals of Oncology*. 2019; 30, 12, 1884-1901 [accessed May 2022].

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Peach H, Board R, Cook M, et al. Current role of sentinel lymph node biopsy in the management of cutaneous melanoma: A UK consensus statement. *Journal of Plastic, Reconstructive & Aesthetic Surgery*. 2020; 73, 1, 36-42 [accessed May 2022].

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Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

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ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

[Get help](#)